

CENTRO DE SALUD CONDUCTUAL DEL SUR

Behavioral Health Progress Note · Confidential — HIPAA Protected

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o, Patient A.	DOB	03/14/1985 (Age 41)	MRN
0/2026	VISIT TYPE	Telehealth (Video)	POS
María E. González, MD	PROVIDER NPI	1457823690	PAYER / PLAN
ON FILE — Pending	TELEHEALTH CONSENT	Signed 04/10/2026	SESSION LENGTH

DIAGNOSES

#	ICD-10-CM	Description	Type
1	F32.1	Major depressive disorder, single episode, moderate	Primary
2	F41.1	Generalized anxiety disorder	Secondary

S — SUBJECTIVE

Patient presents via secure video telehealth platform (Doxy.me) for follow-up individual psychotherapy. Patient reports persistent low mood over the past two weeks, difficulty concentrating at work, and early morning awakening approximately 3–4 times per week. Denies suicidal ideation, homicidal ideation, or active hallucinations. Reports mild improvement in anxiety symptoms since last session.

Patient states: *"I feel a little better about the anxiety but the sadness is still there in the mornings."* Social support remains limited; patient lives alone and reports minimal contact with family in Ponce.

O — OBJECTIVE

Mental Status Exam: Alert and oriented ×4. Appearance appropriate; grooming adequate via video. Speech: normal rate and rhythm. Mood: "sad." Affect: constricted, somewhat reactive. Thought process: linear and goal-directed. Thought content: no delusions. Perception: denies AH/VH. Cognition: intact to interview. Insight: fair. Judgment: fair. Impulse control: adequate.

PHQ-9 score: 14 — Moderate depression (documented in chart). | **GAD-7 score:** 9 — Mild–moderate anxiety (documented in chart).

A — ASSESSMENT

Patient continues to meet diagnostic criteria for **Major Depressive Disorder, Single Episode, Moderate (F32.1)** and **Generalized Anxiety Disorder (F41.1)**. Partial response to psychotherapy. PHQ-9 improved from 17 at intake to 14 today. GAD-7 stable. Ongoing psychosocial stressors (employment, social isolation) contributing to symptom persistence.

P — PLAN

1. Continue weekly individual psychotherapy using Cognitive Behavioral Therapy (CBT) techniques, focusing on behavioral activation and worry management protocols.
2. Medication management review conducted today: current SSRI (Escitalopram 10 mg QD) dosage adequate; no changes made. Patient tolerating medication well with no reported side effects.
3. Referral to group therapy discussed — patient receptive, will consider for next cycle.
4. Safety plan reviewed; patient has active safety plan on file. No acute risk identified.
5. Return for follow-up in one week. Authorization renewal to be initiated by billing office prior to next visit.

Time spent in psychotherapy: 60 minutes (55 minutes face-to-face therapeutic contact + 5 minutes documentation). More than 50% of encounter dedicated to psychotherapy.

Medical decision-making (E&M): Moderate complexity. Two chronic conditions reviewed, medication reconciliation performed, one prescription reviewed. Independently documented and medically necessary — separate and distinct from the psychotherapy service above (Modifier 25 applies).

Electronically signed:

Dr. María E. González, MD

Psychiatrist · License PR-MED-08821 · NPI 1457823690

Date & Time:

06/20/2026 — 14:32 AST

Signed via EHR — RevenueMD Demo System

DEMO RECORD — FOR TESTING PURPOSES ONLY — NOT A REAL PATIENT RECORD — RevenueMD Pre-Submission Intelligence